



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : امال محمود يسن خليل
المحافظة : القليوبية الحي / المدينة : الخانكة
رقم البطاقة : 27007032800226 رقم التليفون : 01203360429
العنوان : ///
العمر (تاريخ الميلاد) : 03-07-1970 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 20-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام اشعة مقطعية رقبة وصدر مع صبغة [11127] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :
قسم :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: امال محمود يسن خليا
National ID: 27007032800226



تقرير طبي

السيد

تحية طيبة وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بان نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الدخول :
--:--:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Contrast-enhanced CT of the neck and chest.

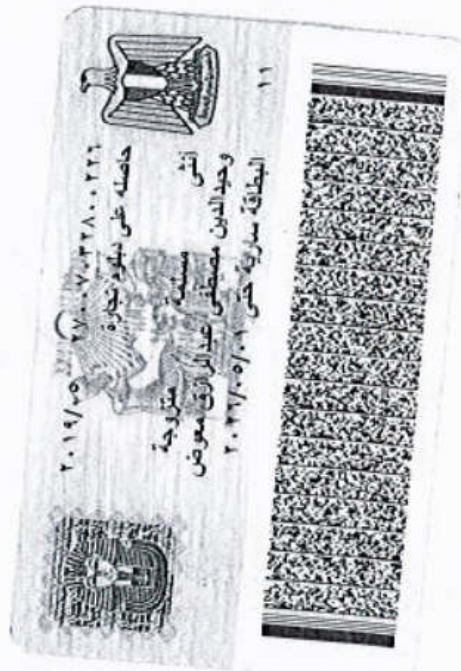
الوقت :

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التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين





Patient Name: Amaal Mahmoud Ysn Khalil	Date : 17/02/2024
Exam: C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 53 Year(s)
Medical ID:	46984
	Total DLP (mGy.cm):688

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED: -**

1. Chest:

- Left breast irregular speculated enhanced mass lesion measuring 3.1 x 3 cm.
- Left lower lobe anterior segment fissural based ill defined reticulo-nodular density is seen.
- Left upper lobe and lingular thick parenchymal bands.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is average in size with homogenous CT texture. No definite focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

3. The scanned skeleton:



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3. Musculoskeletal:



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



MDT form

A-Patient ID:

1- Name : أمال محمود يسن خليل

Sex:

3- Medical number:
Female

83195817

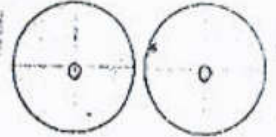
Old Med :

OU Med :

911-2024

2-ID number:

Patient Age : 54 Y



Family history	Family History	Phone	Phone	BMI	BMI	REASON OF MDT	REASON OF MDT
marital status	Marital Status	NRS Score	NRS Score	GROUP	GROUP	Date of MDT	Date of MDT
Co morbidity	Co Morbidity	Menopausal status	Menopausal Status	Initial STAGE	Initial STAGE	24/10/2024	
	htn		POSTMENOPAUSAL				

Clinical data					
Breast Imaging :			Mets Workup :		
			FREE		
BX IDC BX COMMENT:	ER 8/8 ER COMMENT:	PR 8/8 PR COMMENT:	Her 2 Neu 2 Her 2 Neu COMMENT:	SISH Amplified SISH COMMENT:	Ki67
T 2		N 1		M 0	

Sonomammography

- Normal right breast study (BIRADS 1)
- Left LIQ highly suspicious solid lesion with ipsilateral single pathological axillary lymph node. For histopathological correlation (BIRADS 4c)

PATIENT HISTORY

pt received NACT AC x4 + 4 THP
O/E NON PALPABLE MASS NOR LN.
LT WLE+sentinel: IDC GII YPT2(2.5CM), N0/7
free all resection margins

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

- 1- ADJUVANT KADCYLAX12
- 2- PORT
- 3-ADJUVANT AIX5

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY أ.م.د. / 2	RADIOLOGY	RADIOLOGY أ.م.د. / 10
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition



Patient Name: Amaal Mahmoud Ysn Khalil	Date : 02/09/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 54 Year(s)
Medical ID: 911/2024	Pat. ID: 46984

**BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION
ULTRASOUND**

Indication:

Left breast lump

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO views DBT (Digital Breast Tomosynthesis).
- Bilateral breast ultrasound

Report:

- The breasts are Scattered fibro glandular density (ACR B)
- Regressive course of the left LIQ spiculated lesion of high density harboring amorphous microcalcifications measuring 45 X 22 mm seen 3.1 cm from the nipple. Complementary US revealed residual area of distortion Left 9:00 spiculated heterogeneous solid lesion measuring 3.6x2.1 cm
- No right suspicious mass lesions, suspicious micro calcifications or architectural distortion detected.
- Complementary ultrasound showed no right solid or cystic mass lesions.
- Normal right skin thickness and nipple-areola complex.
- Right Nonspecific axillary LNs.
- Non appreciated forenoted left single level I axillary lymph node showing focal cortical thickening and preserved hilum

Opinion:

- Normal right breast study (BIRADS 1)
- Regressive course of the left LIQ highly suspicious solid lesion as well as ipsilateral single pathological axillary lymph node. Representing partial therapeutic response. (BIRADS 6)

Dr. mohamed Mohsen, M.D.

Patient Name : الأستاذة / امال محمود ياسين	Request No. : 13001442
Patient No. : 27007032800226	Request Date : 18/09/2024
Path No. : 3508/24	Verification Date : 07/10/2024
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 54 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data:

Left breast cancer post NACT.

Nature of the specimen:

Left wide local excision and sentenil lymph node biopsy specimens , collected at 18/9/2024.

Gross Examination:

Two specimens were recieved :

- Sentenil lymph node biopsy specimen : Fibrofatty tissue peice measured 4x3x2 cm , enclosing 7 nodules up to 1.5x1 cm in diameter.
- Left wide local excision specimen measured 9x7x2 cm with covering skin ellipse measured 6x4 cm.
- Tumor extent
 - o Number: An ill defined rubbery greyish white tumor bed site (wired and clipped) .
 - o Size: 2.5x2x1cm.
 - o Skin involvement: Free grossly.
- Distance from margins:0.5 cm from inferior margin (least margin).
- Frozen section:
 - Free lymph nodes :(0/7) .
 - Free margins.

Microscopic examination:

- Histological type:Invasive duct carcinoma.
- Grade (differentiation+ nuclear pleomorphism + mitosis): G II(3+2+1).
- Tumor cellularity: 15%
- Angioinvasion: Not detected
- Ductal carcinoma in situ: Minimal DCIS 1%.
- Lymph node involvement:Positive (0/7) .

Verified by :

dr.esraa magdy



Patient Name	: الأستاذة / أمال محمود ياسين	Request No.	: 13008737
Patient No.	: 27007032800226	Request Date	: 21/02/2024
Path No.	: 684/24	Verification Date	: 05/03/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 54 Y	Department	: Out Patient
Referring Doc. :		IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 20/2/2024, 12:20 pm.

Gross Examination:

Four grayish white tissue cores up to 2 cm, totally submitted.

Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4E5), Ki67 clone (30-9) & SISH.

Microscopic Examination:

Histological type: Invasive duct carcinoma.

Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.

Ductal carcinoma in situ: Focal DCIS.

Perineural invasion: No.

Angioinvasion: No.

IPT (Positive control showed appropriate reactivity):

ER: Strong nuclear reaction in 80% of tumor cells (score 8/3).

PR: Strong nuclear reaction in 80% of tumor cells (score 8/3).

Her2: Moderate complete membrane reaction in >10% of tumor cells (score 2+).

Ki67: 17% nuclear reaction of tumor cells.

SISH: Amplified with focal clusters.

Average Her2 copy No. > (6), Her2/CEP17 ratio > (4))

Diagnosis:

Left breast mass, Tru-cut biopsy:

Invasive duct carcinoma, Grade II with focal DCIS.

PT:

ER: Positive (score 8/8).

Verified by :

DR.Menna Nabil

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Page 1 of 2

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Patient Name: Amaal Mahmoud Ysn Khalil	Date : 29/07/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 55 Year(s)
Medical ID: 911/2024	Pat. ID: 46984

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

- Known case of left breast cancer.

I - Chest:

- Multiple pathologically enlarged left axillary lymph nodes largest measuring 24 x 16 mm for sonomammography.
- Right lower lobe fine subpleural reticulations.
- No sizable pulmonary nodules, masses, areas of ground glass attenuation, consolidation or cavitary lesions.
- No CT evidence of pathological mediastinal lymph node enlargement.
- No pleural or pericardial collection, pleural thickening or masses.
- No cardiac gross enlargement.
- Left small elastofibroma dorsi.

II - Abdomen & pelvis:

- The liver is enlarged showing homogenous parenchymal attenuation. No sizable focal lesions.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, kidneys and suprarenals glands.
- Average filled urinary bladder showing no sizable diverticular outpouching.
- Few subcentimeteric para-aortic lymph nodes.
- No ascites.

III - Bone Survey:

- Multiple axial and appendicular skeleton lytic bone lesions likely metastatic.
- Right iliac bone small sclerotic focus.

Dr. Sherief Wagdi, MSc.

Patient Name : الأستاذة / امال محمود ياسين	Request No. : 13001442
Patient No. : 27007032800226	Request Date : 18/09/2024
Path No. : 3508/24	Verification Date : 07/10/2024
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 54 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

- Resection Margins: Free all resected margins.
- Histological response to therapy: Pathologic partial response.

Diagnosis:

Left breast cancer post NACT, wide local excision and sentenil lymph node biopsy specimens:

- Invasive duct carcinoma , grade II.
- Tumor cellularity:15%.
- Free all resected margins.
- Free lymph nodes : (0/7) .
- Pathologic partial response.
- Stage : ypT2 , ypN0.
- RCB:1.717 ,class:II.

Prof.Gamal Abdelatty
Dr.Menna Nabil

Verified by :
dr.esraa magdy

Printing Date : 08/10/2024 08:25:31

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غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

CAP
ACCREDITED



خطابات أورام

Patient Name: Amal Mahmoud Yassen

Date: 4/5/2024

^{99m}Tc -MDP BONE SCAN

Desired information: *skeletal surveying to detect multiple lesions.*

Technique: Anterior and posterior whole-body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc -MDP:

Report:

- Degenerative changes are noticed over Dorso-lumbar vertebrae.
- No definite hot spot that accounts for osseous deposits could be detected all over the surveyed skeleton.
- Osteoarthritic changes are seen at both shoulders , both knees
- Both kidneys are normally seen.

Conclusion:

The surveyed skeleton is free of any active osseous lesions.

Thanks for referral

Nuclear Medicine Unit

DR. Sherin Mahdy, M.D

DR/ Mohamed Emam



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- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.
- Osteopenic texture of the examined bones with advanced dorso-lumbar spondylosis.
- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

- ***Left breast malignant looking mass lesion as described.***
- ***Left lower lobe anterior segment fissural based ill defined reticulo-nodular density likely inflammatory for follow up.***
- ***No hepatic or osseous metastatic deposits.***

Dr. Wafaa Ali, MSc.

Dr. Amr Farouk, M.D.

Patient Name الأستاذة / أمال محمود ياسين
Patient No. 27007032800226
Path No. 684/24
Sex Female
Age 54 Y
Referring Doc.

Request No. 13008737
Request Date 21/02/2024
Verification Date 05/03/2024
Financial Category اورام دار السلام - نفقة الدولة
Department Out Patient

Registered by .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

ER: Positive (score 8/8)
Her2: (score 2+).
FISH: Amplified with focal clusters.
Ki67: Positive in 17% of tumor cells.

Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, 1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 7%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Prof. Dr. Lubna Omar.
Dr. Toka Medhat.



Verified by :
DR.Menna Nabil

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Page 2 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt